# Round Rock Regional Hospital, Department of Radiology

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## DIAGNOSTIC IMAGING REPORT

| | | | |
|---|---|---|---|
| **PATIENT** | WASHINGTON, Jamal | **DOB / AGE / SEX** | 02/14/1979 / 47 / Male |
| **MRN** | RRR-3381920 | **ACCESSION** | RRR-26-1011-0563 |
| **ORDERING PROVIDER** | Kimberly Shaw, DO, Round Rock Regional Emergency Department | **EXAM DATE** | 10/11/2026 10:18 PM |
| **INSURANCE** | Bluebonnet Health, Bluebonnet PPO 500 | **MEMBER ID** | BBH-6025-7713 |
| **REPORT STATUS** | Final | **REPORTED** | 10/11/2026 11:02 PM |

**EXAM:**

CT ANGIOGRAPHY CHEST (PULMONARY EMBOLISM PROTOCOL)

**CLINICAL INDICATION:**

Acute shortness of breath and right-sided pleuritic chest pain 2 days after a 4-hour flight. Elevated D-dimer.

**TECHNIQUE:**

CT angiography of the chest was performed after intravenous administration of 75 mL iohexol 350 using a pulmonary embolism protocol. Multiplanar and MIP reformats were reviewed. CTDIvol: 9.8 mGy. DLP: 352 mGy-cm.

**COMPARISON:**

None available.

**FINDINGS:**

**Pulmonary arteries:** Adequate opacification of the pulmonary arteries. No filling defect to the subsegmental level. Main pulmonary artery measures 2.7 cm.

**Thoracic aorta:** The ascending aorta measures 4.4 cm at the level of the right pulmonary artery. No dissection or intramural hematoma. The aortic arch and descending thoracic aorta are normal in caliber.

**Heart:** Heart size is normal. No pericardial effusion.

**Lungs and airways:** Mild dependent atelectasis in both lower lobes. No consolidation. No suspicious pulmonary nodules. Central airways are patent.

**Pleura:** No pleural effusion or pneumothorax.

**Mediastinum and hila:** No lymphadenopathy.

**Upper abdomen:** 5 mm hypodensity in hepatic segment 6, too small to characterize. Otherwise unremarkable.

**Bones:** No acute osseous abnormality.

**IMPRESSION:**

1. No pulmonary embolism.
2. Dilated ascending aorta measuring 4.4 cm. Recommend annual follow-up imaging with CT angiography or MR angiography.
3. 5 mm hepatic hypodensity, too small to characterize. In a patient without known malignancy, no follow-up imaging is needed.
4. Mild dependent atelectasis.

Electronically signed by: Olivia Grant, MD
Signed: 10/11/2026 11:02 PM
Interpreting facility: Round Rock Regional Hospital



SYNTHETIC TEST DOCUMENT. Fictional patient. Hackathon prototype, not medical advice.